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Illinois Criminal Justice Information Authority
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COMMUNITY-BASED CORRECTIONS TASK FORCE REPORT



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A REPORT TO THE ILLINOIS GENERAL ASSEMBLY

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INTRODUCTION

The Community-Based Corrections Task Force Act (20 ILCS 4134) was passed on August 8, 2024, and signed into law as Public Act 103-0982. The Act created the Community-Based Corrections Task Force, which was charged with developing strategies to introduce community-based corrections and rehabilitation into the Illinois correctional system. The goal was to create a program that would support the advancement of and remove barriers to community-based corrections with pretrial services and services for those sentenced to probation.

As stated in the Act, the Task Force had the following objectives (20 ILCS 4134/10):

- Engage community organizations, interested groups, and members of the public for the purpose of assessing:
 - Community-based alternatives to detention and the adoption and implementation of such alternatives; and
 - The benefits of specialty courts in rehabilitating justice involved individuals.
- Review available research and data on the efficacy of community-based alternatives to detention at the local, state, and national levels.
- Make recommendations or suggestions for changes to the Code of Criminal Procedure of 1963, the Unified Code of Correction, and other relevant statutes.

Additionally, the Act required the Task Force to meet at least five times.

MEMBERSHIP

The Task Force consisted of 29 members (Table 1). Senator Michael Halpin, who sponsored the legislation creating the Community-Based Corrections Task Force Act, served as chair.

As required by the statute, members included the following, as shown in Table 1 below:

Table 1: Community-Based Corrections Task Force Membership List

Appointment	Name	Title	Agency/Organization
Member appointed by the Senate President, Member of the Senate, to serve as Chair of the Task Force	Senator Michael Halpin	Senator	Illinois State Senate
Member appointed by the Senate President, member of the public	Andy Boswell	Probation Officer	Rock Island County
Member appointed by the Senate President, member of the public	Alicia Gomez	Justice-Informed Care Coordinator	Eastern Iowa Mental Health and Disabilities Services
Member appointed by the Speaker of the House, Member of the House	Representative Kelly Cassidy	Representative	Illinois House of Representatives
Member appointed by the Speaker of the House, Member of the House	Representative Gregg Johnson	Representative	Illinois House of Representatives
Member appointed by the Speaker of the House, member of the public	Gina Smith	Crisis Counselor	UnityPoint Health Robert Young Center
Member appointed by the Speaker of the House, member of the public	Mollie Theis, CADC	Program Director	Rock Island County Council on Addictions

Appointment	Name	Title	Agency/Organization
Member appointed by the Minority Leader of the House of Representatives as a member from the House	Representative Patrick Sheehan	Representative	Illinois House of Representatives
Member appointed by the Minority Leader of the House of Representatives, member from the public	Jill Myers	Director of the School of Law Enforcement and Justice Administration and Criminal Justice Professor	Western Illinois University
Member appointed by the Minority Leader of the House of Representatives as a member from the public	Lori Wiese McKee	Legislative Assistant to Jill Tracy	N/A
Member appointed by the Prisoner Review Board	Matthew Coates	Board Member	Prisoner Review Board
Member appointed by the Illinois Criminal Justice Information Authority	Mary Ann Dyar	Program Director, Adult Redeploy Illinois	Illinois Criminal Justice Information Authority
Member from a statewide organization that represents public defenders appointed by the State Appellate Defender	Rebecca N. Klein	First Assistant Public Defender	Rock Island County Public Defender's Office
Member who represents problem-solving courts appointed by the Administrative Office of the Illinois Courts	Dan Hunt	Director of Probation Services	Administrative Office of the Illinois Courts

Appointment	Name	Title	Agency/Organization
Member who represents an organization that provides reentry services appointed by the Department of Corrections Parole Division	Teny Oded Gross	Chief Executive Officer	Institute for Nonviolence Chicago
Member appointed by the Governor's Office of Management and Budget	Kyle Freeman	Deputy Director of Public Safety	Governor's Office of Management and Budget Office
Graduate of a specialty court appointed by the Governor	Morgan Cornette	Self	Self
Graduate of a specialty court appointed by the Governor	Charles Sedam	Self	Self
Graduate of a specialty court appointed by the Governor	Tori Andrews	Self	Self
Graduate of a specialty court appointed by the Governor	Billie Vickers	Self	Self
Graduate of a specialty court appointed by the Governor	Justin Willis	Self	Self
Graduate of a specialty court appointed by the Governor	Jeff McFadden	Self	Self
Graduate of a specialty court appointed by the Governor	Magdalena Campos	Self	Self

Appointment	Name	Title	Agency/Organization
Retired specialty court judge appointed by the Governor	Honorable Mark L. Shaner	Former Drug Court Judge	Self
Retired specialty court judge appointed by the Governor	Honorable Jeffrey Ford	Former Drug Court Judge	Self
Executive Director of the Illinois Sentencing Policy Advisory Council, or his or her designee	Victoria Gonzalez	Executive Director	Illinois Sentencing Policy Advisory Council
Member who represents downstate courts appointed by the Governor	Shay Meredith	Public Defender	Rock Island Public Defender's Office

MEETINGS

The Task Force met on the following dates:

- March 10, 2025
- April 25, 2025
- June 25, 2025
- September 10, 2025
- October 29, 2025
- December 17, 2025
- April 22, 2026

Task Force meeting discussions focused on understanding its duties and perspectives, voting to approve guiding principles, and inviting guest speakers to present on Illinois' community-based corrections landscape and national and state best practices in the field. The presentation described successful programs, system gaps, and the lived experiences of problem-solving court¹ graduates in the community-based corrections process.

¹ Problem-solving courts (also known as specialty, therapeutic, and treatment courts) include drug, DUI, mental health, and veterans courts, which provide intensive supervision and community-based treatment in lieu of incarceration for justice-involved individuals with serious behavioral health issues.

BACKGROUND

Community-based corrections is universally defined as an alternative to traditional institutional corrections that exact punishment, like incarceration in jails or prisons (NIJ, n.d.). It aims to ensure accountability and to support rehabilitation in response to crime. Nationally, there has been a broad effort to establish best-practice standards for core components in the community-based corrections field, such as problem-solving courts by All Rise for drug, DUI, mental health, and veterans courts, and community supervision for probation and parole by the American Probation and Parole Association. See Appendix for more information on best practices.

Illinois has a long history of implementing effective community-based corrections as an alternative to incarceration. Since its creation in 1979, the Probation Services Division of the Administrative Office of the Illinois Courts (AOIC) has created standards and provided technical assistance to guide the development of pretrial services, probation, and problem-solving courts. New initiatives emerged with the passage of the Crime Reduction Act of 2009, which created Adult Redeploy Illinois (ARI) at the Illinois Criminal Justice Information Authority to expand community-based alternatives to incarceration by providing grants to local jurisdictions to reduce recidivism. Through ARI's investments and AOIC's efforts, Illinois has made significant progress at the state and local levels towards evidence-based practices that allow individuals to be supervised in the community rather than prisons, driven by the need for cost-effective rehabilitation.

However, recent changes in legislation, policy, and demographic trends have highlighted the need to continue assessing Illinois' community-based corrections system to ensure it responds effectively to communities' needs. As a result, the Task Force sought presentations on successful existing programs, systemic gaps, and the lived experiences of problem-solving court graduates in the community-based corrections process.

FINDINGS

Testimonies of Current Community-Based Corrections System in Illinois

To begin the Task Force's work, members explored existing programs and initiatives in the community-based corrections field.

First, subject matter experts presented evidence-based principles and programming for community-based corrections. Mary Ann Dyar, Program Director of Adult Redeploy Illinois (ARI) at the Illinois Criminal Justice Information Authority and a member of the Task Force, provided an overview of the community-based corrections programs, including types of community-based corrections and principles of effective interventions. Dyar highlighted the work of ARI, which currently supports 28 grantee sites across 45 counties in the state and operates 60 local prison diversion programs, including problem-solving courts, intensive supervision probation with services, and other system enhancements designed to address client needs and reduce recidivism.

Dan Hunt, Director of Probation Services for the Administrative Office of the Illinois Courts (AOIC), discussed implementing effective probation practices across the state and the development of the statewide problem-solving court standards in 2015 (updated in 2019). The standards established best practices for specialty or therapeutic courts (like drug, DUI, mental health, and veterans courts) that seek to provide a community-based, treatment-focused alternative to prison for individuals in the criminal justice system who have behavioral health, mental health, or substance use disorders (AOIC, 2019).

In addition, Michelle Hart, Project Director at the Center for Advancing Justice at All Rise (formerly the National Association of Drug Court Professionals), discussed the Adult Treatment Court Best Practice Standards at the national level. These standards aim to provide guidance on how best to serve and support adults in treatment court services, ensuring that they are not over-conditioned while also receiving the necessary resources to progress towards their goals. Overall, Hart provided a national perspective on how promising practices in problem-solving courts can inform service innovations for people with behavioral health issues at other points in the system, including at the pretrial stage.

These presentations were followed by a panel discussion with practitioners on strengths and areas for improvement in Illinois' current community-based corrections system. Panelists included:

- David Searby, Perry County's State's Attorney's Office.
- Rebecca Klein, Rock Island County's Public Defenders Office.
- Tammy Williams, Kane County Court Services.
- Mollie Thies, Rock Island County Council on Addictions.

Strengths of the Illinois Community-Based Corrections System

The panelists discussed a variety of initiatives with promising results, including problem-solving courts (also known as specialty, therapeutic, or treatment courts) and other diversion programs, pretrial referral processes and service linkages, and collaboration with referral partners within the current system.

David Searby discussed the Perry County Drug Court and an early diversion program operating in the county. Participants enter the program after they have been charged. Program staff then identify various programs that could provide participants with the resources they need to better their lives, such as employment or counseling. This year-long program has helped community members and has reduced recidivism rates in Perry County.

Focusing on first-offense diversion programs, Rebecca Klein highlighted the benefits of the Offender Initiative Program, Section 70 Probation, and Section 410 Probation. These programs serve people with no prior convictions and provide resources to help them avoid recidivism. By taking part in one of these diversion programs, participants can avoid a felony conviction.

Other initiatives with preliminary positive results include pretrial referral processes following the elimination of cash bail. As Tammy Williams detailed, Kane County pretrial officers connect participants with resources before their trial starts, not after a plea deal or conviction. This shift has enabled participants to receive much-needed referrals and service linkages sooner rather than later. Williams also mentioned anecdotally that the elimination of cash bail has led to positive changes in her area, making the pretrial process more equitable and reducing the likelihood that individuals with non-violent offenses will become violent while held in custody.

Williams noted that, since the elimination of cash bail, failure-to-appear rates have declined in Kane County.

Additionally, local efforts that prioritize person-centered care rather than a “one-size-fits-all” approach have resulted in positive changes for justice-involved clients. Mollie Thies, from the Rock Island County Council on Addictions, reported that, in the referrals she has seen, a focus on individualized services and client well-being has led to the provision of necessary resources and an appropriate level of care to disrupt the cycle of crisis, crime, and punishment.

Challenges

Panelists also identified barriers to success in community-based corrections programming, especially in rural communities. Panelists reported that many rural communities lack access to treatment and other human services, as well as to housing and reliable transportation. Given these communities’ resource constraints, participants may not be able to access services and supports to address their needs and reduce their risk of recidivism. Even for clients who can access services outside their communities, location prevents them from being close to services during a crisis. Panelists discussed Iowa’s halfway housing program as a potential option for the Task Force to consider. However, Task Force members highlighted that Illinois has existing halfway houses, but clients in Illinois still face housing access as a regular barrier to treatment.

As Williams noted, pretrial officers encounter the same issues connecting clients to local services, even in a populous area like Kane County. Additional barriers include long wait times for resources (sometimes upwards of six months in Kane County) and insufficient tailoring of services to individualized risk and needs. Advocating for tailored services, Williams noted that some individuals do not need intensive supervision by community-based corrections officials and that community-based interventions can better support their development. Absent this tailoring, however, some participants may become over-conditioned and given unnecessary supervision from community-based corrections officials.

A final barrier discussed by the panel was insufficient support for mental health. Many participants may have mental health challenges, and increased crisis intervention program would be able to de-escalate situations in which participants were combative with other officials, such as law enforcement or hospital staff.

Lived Experience within the Community-Based Corrections Process

To build upon evolving findings, the Task Force invited Magdalena Campos and Jeff McFadden to discuss their experiences as problem-solving court clients and graduates. The goal was to understand the lived experiences of people who participate in problem-solving courts, the beneficial support they receive from the courts, and opportunities to improve and further strengthen the courts. Campos and McFadden emphasized a positive overall experience with the problem-solving courts, noting that the incentives-and-sanctions structure, along with dedicated staff, provided critical support that improved their lives. As a possible enhancement, both panelists suggested introducing peer support roles into court care teams, enabling participants to connect with graduates who share their experiences and to help empower them during their problem-solving court process. As a whole, the panelists' experiences demonstrated that Illinois' problem-solving courts are an effective means of improving the lives of individuals involved in the community-based corrections process.

Programming to Address Illinois' Gaps

To identify potential ways to address the existing gaps in Illinois' community-based corrections system with programming, the Task Force invited the following speakers:

- Scott Block, Administrative Offices of Illinois Courts (AOIC).
- Stephanie Frank, Illinois Division of Behavioral Health and Recovery (DBHR).
- Lori Carnahan, DuPage County Crisis Recovery Center (CRC).
- Chris Koepke, Iowa's Davenport County Co-Responder Program.
- Christina Fyfe, Iowa Department of Corrections' Residential Services.

Active Programming in Illinois

Scott Block, Statewide Behavioral Health Administrator for AOIC, detailed the work of the Illinois courts in preparing a mental health action plan. As part of that effort, Block described the use of the Sequential Intercept Model (SIM) in regional workshops. When used as an evaluation tool, the SIM assesses local resources for people with behavioral health needs, identifies gaps in criminal

justice processes, and aligns systems to create comprehensive behavioral health and justice programming. Specifically, Block highlighted the dynamic picture the SIM provides of the methods and systems that people encounter as they move through the criminal justice system. It identifies the resources, services, gaps, best practices, and evidence-based practices implemented in a given county's criminal justice system. The SIM helps government officials and other system stakeholders identify strengths and opportunities for growth through collaboration and innovation. Findings from previous SIM evaluations in Illinois have led to the development of critically needed resources, as seen in the DuPage County Crisis Recovery Center.

Stephanie Frank, Associate Director of Planning and Transformation at the Illinois Division of Behavioral Health and Recovery (DBHR), provided an overview of community crisis and deflection efforts within her division and, more broadly, in the Department of Human Services (DHS). She discussed the implementation of the Community Emergency Services and Supports Act (CESSA) across the state. She described programs such as Illinois' statewide 988 mobile crisis hotlines, crisis response teams, and crisis facilities, all of which serve to resolve behavioral health crises without police intervention. The DBHR has found that these measures have resulted in decreased ambulance, hospital, and jail usage across Illinois. CESSA redirects 911 emergency calls to behavioral health providers when law enforcement involvement is not necessary. The implementation of CESSA is ongoing, and Frank reported that current efforts have been successful in connecting community members in crisis to essential services. Finally, Frank outlined the extensive deflection efforts that DHS has developed. Working closely with the Illinois State Police to build county-level connections, DHS has expanded deflection programming across the state. However, Frank noted that not every county has a deflection program, with many southern counties lacking one.

Lori Carnahan, Deputy Executive Director of the DuPage County Health Department, discussed treatment-based crisis response services available at the new Crisis Recovery Center (CRC). The CRC was created to redirect high-level crisis responses, like mental health-based emergency room visits, by providing immediate 24-hour, seven-day-a-week services to individuals facing mental health or substance use crises. The CRC offers participants a safe, walk-in space, evaluations, stabilization, and connection to long-term care. While an individual is in care, the CRC assists with medication management, crisis intervention, psychiatric assessment, crisis counselors, peer support staff, safety planning, and medically assisted treatment for substance use stabilization. At the time of presentation, the CRC had been open for three months and had already conducted 300 adult

assessments and 72 youth assessments. Only 3% required a high level of care, such as an emergency department visit. Overall, the CRC shows potential as a model for treatment-based crisis response services within the community-based corrections system.

Neighboring States' Additions to the Community-Based Corrections Process

To further expand on beneficial models, the Task Force sought to learn more about promising models in Iowa. Chris Koepke, an experienced mobile crisis co-responder, described the process Davenport County responders use for mental health-related incidents reported to the police department. In this process, one police officer and one social worker respond, de-escalate the incident, and connect citizens to the resources they need to improve their mental health and well-being. Koepke also emphasized the team's focus on proactive outreach. They contact high utilizers of 911 and connect them with referral sources or community partners who can better address their needs. As a result of de-escalation and resource connection, 624 cases (44.7% of all encountered cases) were successfully de-escalated with no further need for police intervention. Looking forward, Koepke advocated for increasing the mobile crisis response team to five police officers and five mental health workers to cover the entire county. He also advocated for 24/7 coverage and for covering substance use cases.

Christina Fyfe described operations and support processes in Iowa's 21 statewide residential correctional facilities. Iowa has about 1,647 available beds for residential clients distributed across eight districts. Residential officers monitor client accountability across all facilities, and in the communities, case managers and support staff handle administrative duties. To become a client, individuals must be sentenced by a district court or the Iowa Board of Parole. Clients are required to participate in services related to their assessed needs and experiences. Client services, for example, may be cognitive behavioral therapy, mental health treatment, substance abuse treatment, an impaired driver accountability program, or a sex offender treatment program. Staff in residential facilities help clients access treatment opportunities by providing referrals. Another requirement is that clients must secure full-time employment, which is facilitated by staff. Staff connect clients to community partners, and they provide support for budget planning, release planning, and transportation to employment.

RECOMMENDATIONS

Programming Recommendations

The Task Force identified the need to uncover gaps within county community-based corrections systems and leverage best practices and local and state resources to expand services. The Task Force's recommendations included:

- Conducting Sequential Intercept Model (SIM) evaluations.
- Creating criminal justice coordinating councils.
- Conducting two pilot assessments.
- Expanding deflection and community crisis programs.
- Studying problem-solving court expansion.
- Creating statewide standards.
- Identifying areas for further research on Illinois' community-based corrections system.

Recommendation #1: Conduct a Sequential Intercept Model (SIM) Evaluation in each county or circuit

The Task Force recommended that each county or circuit conduct a SIM mapping exercise and evaluation. Outcomes would create a foundation for better coordination among local stakeholders and address gaps in county-level programming. The Task Force suggested that the General Assembly require each county or circuit to conduct a SIM evaluation within the 2027-2028 Fiscal Year and establish plans to reconduct SIM evaluations every three years. The Task Force recommended that the state provide necessary funding to conduct these evaluations.

Recommendation #2: Create a Criminal Justice Coordinating Council in each county or circuit

The Task Force strongly recommended that the General Assembly require each county or circuit to create a Criminal Justice Coordinating Council. Criminal Justice Coordinating Councils bring

together a county's community-based corrections officials, courts, offices, and agencies to facilitate jurisdictional collaboration (JMI, n.d.). Through collaboration, all stakeholders share information, processes, and procedures within each area of the community-based corrections process. The Task Force recommended that the state provide necessary funding to establish these councils.

Recommendation #3: Pilot a residential correctional facility in Rock Island County and assess the DuPage County Crisis Recovery Center

The Task Force recommended conducting a two-pilot assessment. One pilot would apply Iowa's residential correctional facility model to Illinois. This pilot would be hosted in Rock Island County, provided that the county identifies a need for it after its SIM evaluation. Community-based corrections officials would identify potential candidates on probation or court supervision who would benefit from this pilot residential correctional facility using evidence-based screening and assessment tools. The Task Force also recommended collecting data and other information to understand the successes of and areas for improvement within the residential correctional facility. This information could include, but would not be limited to:

- Participant recidivism rates.
- Available resources that people are connected to.
- Data about adherence to treatment.

The Task Force also recommended an assessment of the newly established DuPage County Crisis Recovery Center to understand its successes and areas in need of improvement. As above, the assessment could include, but would not be limited to:

- Participant recidivism rates.
- Available resources to which people are connected.
- Data about adherence to treatment.

The goal of the two pilot projects is to offer crisis stabilization models for justice-involved individuals with behavioral health needs in Illinois. After conducting the SIM evaluation, counties and circuits could decide whether these models would benefit their areas and help address

service gaps. Information on successes and areas for improvement from these pilot assessments could be shared with counties and circuits, allowing them to tailor their chosen pilot to their specific needs.

Recommendation #4: Expand deflection programming across Illinois and support implementation of CESSA, community crisis programs, and deflection programs

Deflection programs provide a treatment-based alternative to arrest for people in behavioral health crises. According to the DHS, not all counties in Illinois have deflection programs. The Task Force recommended expanding deflection programs statewide. Moreover, given the importance of crisis response and treatment-based approaches across Illinois to address mental health and substance use emergencies, the Task Force also endorsed supporting CESSA implementation, community crisis programs, and deflection initiatives. The Task Force recommended using existing services, when possible, to serve the Illinois community better and prioritizing treatment-based services as an alternative to incarceration.

Recommendation #5: Study the expansion of problem-solving courts' capabilities

The Task Force recommended that an additional study be conducted to explore updated legislation that allows for early release from the Department of Corrections to local problem-solving courts, provided that a participant attends available substance use disorder and/or mental health programming during incarceration. The study would examine problem-solving court practices and establish procedures that align with current policy and best practices. As envisioned by the Task Force, the problem-solving courts would first review potential participants' cases to determine whether participants qualify under the problem-solving courts' policies and procedures for entry into the problem-solving court. The judge and the corresponding problem-solving court team would review eligibility cases in accordance with the problem-solving courts' policies and procedures. If the potential participant is allowed to enter the problem-solving court and agrees to follow all its rules, they would be eligible for early release. However, if the participant violated the rules and the agreement with the problem-solving court, the participant would have to serve their previous sentence.

The additional study to align practice and policy should involve the Department of Corrections, the Prisoner Review Board, Problem Solving Court and Probation professionals, and any other state entities involved in the incarceration process.

Recommendation #6: Create statewide standards and regulations through statute or through updates to problem-solving court standards, or otherwise, to facilitate transfers of problem-solving court cases between counties

The Task Force recommended creating statewide standards and regulations to facilitate formal transfers of problem-solving court cases between counties. As previously mentioned, not every Illinois county has a problem-solving court or sufficient resources to provide the required treatment. Similarly, individuals may be sentenced in one county but live in another. As a result, the formal creation of statewide standards for transferring problem-solving court cases between counties is necessary to ensure that individuals across the state receive optimal levels of care. These standards can be created either through statute or as updates to problem-solving court standards.

Awareness and Training Recommendations

The Task Force identified increased awareness and training as other important areas in need of improvement. Due to the immense variation in services, many community-based corrections officials may not be aware of the services offered and conducted across Illinois that can be beneficial to their clients and communities. For example, increased awareness is needed to improve understanding of DHS resources across the community-based corrections continuum in Illinois. Similarly, the Task Force recommended prioritizing training for officials on treatment-based approaches to the community-based corrections system. Offering additional training to current and future community-based corrections officials would improve and expand the use of treatment-based approaches.

Recommendation #7: Increase Illinois state agencies' awareness of community-based corrections efforts

The Task Force appreciated hearing about community-based corrections initiatives across Illinois, many of which were previously unknown to members. As a result, the Task Force recommended that Illinois state agencies enhance their awareness efforts so that officials and individuals involved in the community-based corrections processes are aware of essential contributions and findings. This would ensure that officials and members of the public are aware of resources in their county and across the state, enabling effective referrals.

Recommendation #8: Expand trainings to focus on treatment-based approaches

The Task Force identified a need to expand treatment-based approaches across the community-based corrections system to better serve people in Illinois. As a result, the Task Force recommended additional training and continuing education programming on treatment-based approaches for community-based corrections officials. These trainings and continuing education opportunities would expand knowledge of best practices and available resources within the community-based corrections workforce, enabling a treatment-based perspective to be prioritized.

Funding Recommendations

Finally, the Task Force identified funding barriers that impede implementation of best practices and involvement in the community-based corrections process. The Task Force acknowledged significant budget pressures for the state, yet still proposed additional funding when possible.

Recommendation #9: Allocate additional funding

The Task Force recommended allocating additional funding, as follows:

- Funding to support local jurisdictions' efforts to facilitate SIM evaluations and CJCC development.
- Funding to create a residential correctional facility pilot and corresponding grant funding for evaluation of the residential correctional facility and Crisis Recovery Center.

- Funding to expand the capacity of treatment facilities.
- Grant funding to support transportation to services for people involved in the community-based corrections system across the state.
- Increased funding specific to rural areas, including but not limited to funding for transportation, expansion of county services, and additional partnerships with neighboring counties.
- Additional funding for problem-solving courts to increase personnel and services to expand capacity.
- Additional grants to support mental health and substance use disorder services.
- Funding to expand deflection and community crisis programming statewide.
- Grant funding to expand the community-based corrections workforce (for example, funds for tuition payments, scholarships, internship stipends, training for treatment-based approaches, or certification-related payments).

CONCLUSION

The Task Force's review of Illinois' community-based corrections landscape identified significant strengths in local innovation and cross-system collaboration, while also highlighting gaps in coordination, resources, and consistent statewide practices. To address these challenges and build on existing successes, the Task Force recommended a set of strategic actions to strengthen the effectiveness and sustainability of community-based corrections across Illinois.

Key recommendations included conducting Sequential Intercept Model (SIM) evaluations in all counties and establishing Criminal Justice Coordinating Councils (CJCCs) to support and guide those assessments; piloting and evaluating a residential community corrections facility; and assessing the DuPage County Crisis Recovery Center to determine feasibility for replication in other regions of the state. The Task Force also recommended expanding deflection programming, supporting the implementation of community crisis initiatives led by the Illinois Department of Human Services, and conducting a statewide study to evaluate opportunities to expand problem-solving courts. In addition, the Task Force recommended developing standardized procedures for transferring problem-solving court cases between counties to improve continuity of care and program access.

To further support these efforts, the Task Force recommended increasing statewide awareness of community-based corrections initiatives implemented by Illinois agencies, expanding training opportunities that emphasize treatment-based and behavioral health-informed approaches, and allocating the resources necessary to implement these recommendations effectively.

Taken together, these actions will help Illinois strengthen coordination among justice, behavioral health, and community service systems; expand access to effective alternatives to incarceration; and ensure that community-based corrections strategies are implemented consistently and effectively across the state. With sustained leadership, collaboration, and investment, Illinois can continue to advance a more responsive, evidence-informed approach that improves public safety, supports rehabilitation, and promotes long-term community stability.

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<https://nij.ojp.gov/topics/corrections/community-corrections>

APPENDIX

Community-Based Corrections Best Practice Materials

Crime and Justice Institute at Community Resources for Justice, *Implementing Evidence-Based Policy and Practice in Community Corrections*

https://www.crj.org/assets/2017/07/Community_Corrections_BoxSet_Oct09.pdf

National Association of Drug Court Professionals, Drug Court Standards Committee, *Defining Drug Courts: The Key Components*

<https://allrise.org/wp-content/uploads/2022/07/Defining-Drug-Courts-The-Key-Components.pdf>

All Rise, *Adult Treatment Court Best Practice Standards*

https://allrise.org/wp-content/uploads/2026/01/All-Rise_Adult-Treatment-Court-Best-Practice-Standards_January-2026.pdf

American Probation and Parole Association, *Standards Development Working Group National Standards for Community Supervision*

https://www.appa-net.org/eweb/docs/APPA/National_Standards_Community_Supervision_FINAL_3-17.pdf

Administrative Office of the Illinois Courts, *Problem-Solving Courts Standards*

<https://drive.google.com/file/d/1pvfw88MDI27cuEFW6ZMDvccuqRZez72g/view>



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